

P&L Pathway Statements — for Mr Jawad's confirmation

17 newly drafted pathway statements (block 05 of the capability pages), live on the Evidence Room since 19 Sep. The blueprint assigns this block to Finance — please tick to confirm, or mark edits. Verified figures only were used where verified; all other statements describe the economic mechanism without claiming measured results.

ID	CAPABILITY	P&L PATHWAY STATEMENT	CONFIRM
1.1	Network Footprint & Capacity	Three operating branches are the revenue engines; capacity — chairs and doctor-hours — is the binding constraint on treatment revenue. Utilisation gains convert fixed clinic cost into margin, and every empty chair-hour is perishable inventory lost.	☐
2.2	Website & Content Infrastructure	The bilingual site and 19,500+-page knowledge base convert organic demand into enquiries the group pays nothing per-click for — every organic booking displaces paid acquisition cost, and the content estate compounds rather than depreciates.	☐
2.3	Marketing Intelligence & Demand Generation	Intelligence spends the media budget where a dirham buys the most treatment value: audience segmentation and quality-tagged lead feedback steer spend toward channels that produce booked patients, not clicks — raising revenue per marketing dirham and cutting waste on unconvertible demand.	☐
2.4	Patient Acquisition	2,910 deduplicated leads YTD (live feed) top the treatment P&L; the governing economics is acquisition cost per booked patient, not per lead — and channel-level attribution lets losing channels be cut before they consume budget.	☐
2.5	Membership, Loyalty & Retention	Membership converts episodic patients into recurring, prepaid relationships: annual fees are cash collected against future care, retention lifts lifetime treatment value, and benefit usage drives visit frequency. Economics are gated on contribution after benefit-delivery cost, measured through the live 30-day activation mandate.	☐
3.1	Contact Centre & Patient Support	Speed-to-answer is revenue protection: every missed or slow-answered enquiry is paid demand lost — rapid qualification converts marketing spend into booked appointments instead of wasted clicks.	☐
3.2	Booking, Conversion & Lead Management	Lead-to-booking conversion is the highest-leverage ratio in the acquisition P&L: at a fixed media budget, each point of conversion is revenue at zero incremental acquisition cost. The 24-hour close rule and per-lead status tracking keep that pipeline honest.	☐
3.4	Follow-up, Recall & Retention	Recalls and follow-up monetise patients already acquired: preventive recalls fill chair-hours at zero acquisition cost, and the follow-up pool re-activates treatment demand the group has already paid to generate.	☐
4.1	Clinical Leadership & Governance	Governance is margin protection: daily branch reporting catches revenue leakage while it is still correctable, and formula-controlled commission reconciliation (August verified to AED 0.02) makes the group's largest variable cost — clinician pay — auditable and dispute-free.	☐
4.2	Standards, Protocols & Care Pathways	Standardised pathways make revenue reproducible: consistent treatment planning and documentation reduce rework, support payer and regulator confidence, and let any clinician deliver the group standard of care — the precondition for adding clinics without margin dilution.	☐
4.3	Quality, Safety, Compliance & Audit	Quality is the cheapest acquisition channel the group has: the 4.9★ public record converts directly into organic demand, while the compliance register (licenses, radiology registration, service contracts with expiry dates) protects the licence to operate — a single lapsed permit can halt a branch's revenue overnight.	☐
4.4	Clinical Training & Competency	Cross-branch training turns one branch's specialist capability into group capability: signed-off procedure observation widens what every branch can safely offer — lifting revenue per chair without new hires — and documented competency reduces clinical and insurance risk.	☐
5.1	Centralized Shared Services	Centralisation converts three overheads into one: one finance feed, one procurement negotiation, one intake process. Unit costs fall as clinics are added — the operating-leverage core of the platform thesis.	☐
5.2	Organization & People	The two-pillar structure keeps clinical and commercial accountability separate but coordinated: clear ownership shortens decisions, and shared roles across branches spread fixed people-cost over more revenue.	☐
5.3	Technology, Data & Automation	Automation replaces manual reporting hours with machine feeds on a 15-minute cadence, and one data spine removes the hidden cost of reconciling conflicting numbers — decisions land days earlier on the same headcount.	☐
5.4	Performance Management	Target-vs-actual pacing on a live feed turns month-end surprises into same-week corrections — underperformance is fixed or cut while the month is still winnable.	☐

5.5 Executive Intelligence
Dashboard

One verified number set for every decision: the dashboard removes the recurring cost of conflicting reports and makes doctor, branch and channel economics visible to the dirham — the lens that lets one management team run three clinics.



Already carrying Finance-relevant figures (previously filled, for reference only): 1.2 doctor payable formula (August AED 126,388, reconciled to AED 0.02) · 1.3 ortho as deepest-value lane · 3.3 unconverted-plan recovery economics · Layer 06 expansion economics.